

Chemical Peel Reference

Peel depths, acids, skin types, pre/post protocols, and contraindications

PEEL DEPTH CLASSIFICATION				
Depth	Layer Reached	Recovery	Common Acids	Best For
Superficial	Epidermis only (stratum corneum to basal layer)	0–3 days flaking	Glycolic 20–50%, Lactic 30–50%, Mandelic 20–40%, Salicylic 20–30%	Texture, mild hyperpigmentation, acne, maintenance, sensitive skin intro
Medium	Papillary to upper reticular dermis	5–10 days peeling	TCA 20–35%, Jessner's + TCA, Modified Jessner's, Glycolic 50–70%	Moderate wrinkles, uneven pigmentation, acne scarring, sun damage
Deep	Mid to lower reticular dermis	14–21 days significant downtime	TCA 35–50%, Phenol, Baker-Gordon	Deep wrinkles, severe sun damage, precancerous lesions — physician only

ACID REFERENCE GUIDE				
Acid	Type	Strengths	Special Properties	Caution
Glycolic (AHA)	Alpha-hydroxy acid	20–70% in practice	Smallest AHA molecule — deepest penetration. Gold standard for anti-aging	Most irritating AHA. Not ideal for sensitive skin or darker Fitzpatrick
Lactic (AHA)	Alpha-hydroxy acid	30–70%	Larger molecule = gentler than glycolic. Also a humectant — hydrates while exfoliating	Good for sensitive skin, dry skin, Fitzpatrick IV–V with care
Mandelic (AHA)	Alpha-hydroxy acid	20–40%	Largest AHA — gentlest penetration. Also antibacterial. Excellent for acne + darker skin	Best AHA for Fitzpatrick IV–VI — lowest PIH risk
Salicylic (BHA)	Beta-hydroxy acid	10–30%	Oil-soluble — penetrates follicles. Anti-inflammatory, antibacterial	Best for acne, oily skin, congestion. Cannot use in aspirin allergy
TCA	Trichloroacetic acid	10–35% (practice)	Strength determines depth. Frosting indicates penetration depth achieved	Medium-deep peels require training. Depth control critical. PIH risk in darker skin
Jessner's	Combination (lactic/salicylic/resorcinol)	Standard formulation	Enhances penetration of subsequent acids. Used as a priming layer before TCA	Resorcinol in formula — check for allergy
Kojic acid	Tyrosinase inhibitor	2–4%	Depigmenting — not exfoliating alone. Often combined with glycolic or lactic	Sensitizer in some patients. Not a standalone peel

Fitzpatrick & Peel Selection					Pre-Peel Protocol (2–4 Weeks Before)	
Fitzpatrick	Skin Description	Peel Risk	Recommended Peels	Avoid	Tretinoin / retinol	Initiate 4–6 weeks pre-peel to thin stratum corneum, improve penetration, speed healing. Stop 3–5 days before peel
I–II	Very fair, burns easily, rarely tans	Low PIH risk	All peel depths appropriate. Aggressive treatment tolerated well	Standard precautions	Hydroquinone (Fitzpatrick IV–VI)	4–6 weeks pre-peel to suppress melanocytes and reduce PIH risk in darker skin types. Essential for medium peels in darker skin
III–IV	Medium, tans gradually, mild burn	Moderate PIH risk	Superficial peels preferred. Mandelic, lactic, low-% salicylic. TCA with caution	High-% glycolic, phenol peels	SPF 50+	Daily broad-spectrum sunscreen throughout pre-peel period. Non-negotiable — UV exposure worsens pigmentation and healing
V–VI	Dark brown to black, rarely burns	High PIH risk	Superficial only. Mandelic preferred. Very low % salicylic. Must prime skin 4–6 weeks first	TCA >15%, all medium-deep peels, phenol	Discontinue	AHAs/BHAs 5–7 days before. Waxing or hair removal 2 weeks before. Retinoids 3–5 days before. Accutane 6–12 months before
					HSV prophylaxis	Antiviral prophylaxis (acyclovir/valacyclovir) for ANY patient with history of oral or facial HSV — start 48 hrs before and continue 5 days post-peel

Post-Peel Protocol	
Days 1–3	Gentle cleanser only. Thick fragrance-free moisturizer (CeraVe, Aquaphor for deep peels). Absolutely no picking, peeling, or scrubbing
Days 4–7	Skin will peel significantly — let it happen naturally. Peeling = expected, not a complication. Continue gentle moisturizer
SPF	SPF 50+ from Day 1 post-peel. No exceptions. UV exposure during healing dramatically increases PIH risk
Avoid	Retinoids, AHAs, BHAs, active ingredients for minimum 7–14 days post-peel depending on depth
No heat	No saunas, steam rooms, strenuous exercise for 5–7 days post-peel — heat increases inflammation and PIH risk

Absolute Contraindications	
✗	Active Accutane (isotretinoin) use or within 6–12 months — impaired wound healing, increased scarring risk
✗	Active skin infection, open wounds, or active herpes outbreak in treatment area
✗	Pregnancy — category C/D for many peel agents
✗	Allergy to specific acid or any component of the peel formulation
✗	Unreliable patient who will not follow post-peel sun protection — PIH guaranteed without compliance
⚠	Recent sun exposure or active tan — significantly increases burn and PIH risk in ALL skin types